



معسكر الورقة الثانية للأطفال — الأسئلة المقالية



Essay Booklet – Day 2 (مقالي اليوم الثاني)



Student Copy

Total Questions: 87 | Second Paper
Camp

BOOKLET TYPE

Essay Questions (مقالي)

TOTAL QUESTIONS

87 Questions

ACTIVE CAMP

Second Paper (الورقة الثانية)

CHAPTER 01

Renal Diseases

HEMATURIA

Q1 Define hematuria and microscopic hematuria

Q2 Enumerate the causes of hematuria in children (glomerular and extra-glomerular)

Q3 How can you differentiate glomerular hematuria from lower urinary tract hematuria?

Q4 Enumerate the causes of dark red / brown urine

Q5 List the laboratory investigations used in the evaluation of a child with glomerular hematuria

Q6

Mention the indications for renal biopsy in a child with hematuria

ACUTE POST-STREPTOCOCCAL GLOMERULONEPHRITIS (APSGN)

Q1 Define APSGN

Q2 Describe the clinical picture of APSGN

Q3 Enumerate the investigations used to confirm the diagnosis of APSGN

Q4 Enumerate the complications of APSGN

Q5 List the medical emergencies in APSGN

Q6

Mention the treatment of APSGN

NEPHROTIC SYNDROME

Q1 Define nephrotic syndrome and mention its four characteristic features

Q2 Describe the pathogenesis of edema in nephrotic syndrome

Q3 Enumerate the complications of nephrotic syndrome

Q4 Describe the steroid therapy used in idiopathic nephrotic syndrome

Q5 List the indications for renal biopsy in nephrotic syndrome

Q6

Mention the indications for IV albumin infusion in nephrotic syndrome

ACUTE KIDNEY INJURY (AKI)

Q1 Define AKI, oliguria, and anuria in children

Q2 Enumerate the causes of AKI classified into prerenal, intrinsic renal, and postrenal

Q3 How do you differentiate prerenal AKI from intrinsic renal AKI?

Q4 Enumerate the clinical manifestations of AKI (oliguric phase)

Q5 List the stages of AKI according to modified pRIFLE criteria

Q6 Mention the electrolyte and acid-base disturbances seen in advanced AKI

CHAPTER 02

Neonatology

Topic 1: Transient Cutaneous Lesions

Q1 Define transient cutaneous lesions in the newborn

Q2 Enumerate the transient cutaneous lesions seen in the newborn

Q3 Define Acrocyanosis

Q4 Define Harlequin color change

Q5 Define Erythema toxicum

Q6 Mention the clinical features of Erythema toxicum

Q7 Define Mongolian spots

Q8 Mention the clinical features of Mongolian spots

Q9 Mention the clinical features of Pustular Melanosis

Q10 Differentiate between Erythema toxicum and Pustular Melanosis

Q11 List four transient skin features or substances related to maturity or delivery found on a newborn's skin

Topic 2: Prematurity and Its Complications

Q1 Define Prematurity / Premature infant

Q2 Enumerate the causes of prematurity

Q3 Enumerate the physiological handicaps of prematurity

Q4 Enumerate the physiological handicaps of the premature infant specifically regarding Respiratory and Gastrointestinal systems

Q5 List four physical features or signs of underdevelopment observed during the clinical examination of a premature infant

Q6 List the Respiratory complications of prematurity

Q7 List the Central Nervous System (CNS) complications of prematurity

Q8 Enumerate the Cardiovascular and Gastrointestinal complications of prematurity

Q9 Mention the Hematologic complications of prematurity

Q10 List four Metabolic and Endocrine complications of prematurity

Q11 Mention the essential guidelines for prophylactic vitamin administration in the management of a preterm infant

Topic 3: Compare Between Physiological and Pathological Jaundice

Q1 Define the Enterohepatic Circulation of Bilirubin

Q2 Define and state the criteria of Physiological Jaundice

Q3 List the predisposing factors/causes that explain the development of physiological jaundice

Q4 Enumerate the risk factors for exaggerated physiological jaundice

Q5 Define Pathological Jaundice and list the criteria used to identify it

Q6 Mention four clinical signs of an "Ill baby" with Pathologic Jaundice

Q7 Enumerate four Pathologic causes of Neonatal Jaundice due to "Increased Formation" (Hemolysis), excluding immune incompatibility

Q8 Enumerate four causes of Pathologic Jaundice due to Conjugation Defects

Q9 Mention four causes of Pathologic Jaundice related to Excretion or Secretion Defects

Topic 4: Complications of Indirect Hyperbilirubinemia

Q1 Enumerate the complications of indirect hyperbilirubinemia

Q2 Define Inspissated Bile Syndrome

Q3 Define Kernicterus

Q4 Enumerate the predisposing factors for kernicterus

Q5 Describe the clinical stages of Acute Kernicterus

Q6 List the clinical features of Chronic Kernicterus (by 3 years of age) and during the 2nd year of life

Q7

Mention four preventable causes/pitfalls leading to Kernicterus

Q8

List the parameters to monitor and evaluate when performing a visual/clinical assessment of a jaundiced newborn

Q9

Enumerate four major complications or risks associated with performing an Exchange Transfusion

Topic 5: Neonatal Sepsis and Its Clinical Sepsis Score

Q1 Define Neonatal Sepsis

Q2 Define Nosocomial Sepsis

Q3 Differentiate between Early onset and Late onset neonatal sepsis

Q4 Differentiate between Possible, Probable, and Proven Sepsis based on diagnostic classification

Q5 Enumerate the risk factors for neonatal sepsis

Q6 List the causative organisms of neonatal sepsis

Q7 Mention the Clinical Sepsis Score for neonatal sepsis

Q8 List four clinical signs of Central Nervous System (CNS) and Gastrointestinal (GI) dysfunction in a septic neonate

Q9 Enumerate the investigations (sepsis work-up) for neonatal sepsis

CHAPTER 03

Family Medicine

Title 7 & 8: Anticipatory care (1 & 2)

Q1 Enumerate the three distinct levels of disease prevention included under Anticipatory Care, along with the core definition or objective of each level

 Case Scenario:

Case 7:

A family physician is conducting a periodic health examination for an adolescent patient. The physician aims to implement systematic, evidence-based measures to promote good health, screen for underlying psychosocial risk factors using the HEEADSSS framework, and carefully verify vaccination histories and clinical baselines.

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Q2 Enumerate the six essential operational elements that a family physician must comprehensively evaluate to build a proper database during a periodic health examination

Q3 Enumerate the specific clinical parameters that correspond to each letter of the ****HEEADSSS**** psychosocial assessment interview framework used during adolescent screening

Q4 List four true clinical contraindications to implementing childhood immunizations in a family practice setting

Title 9: Breast feeding

- Q5** Enumerate the four clinical scenarios where a mother must ****ABSOLUTELY NOT**** breastfeed and ****NOT**** feed expressed breast milk to her infant

 Case Scenario:

Case 8:

A mother presents to the family medicine center for a postpartum consultation. The family physician systematically assesses her medical history to ensure there are no infectious, metabolic, or pharmacological conditions that would serve as an absolute or temporary contraindication to breastfeeding or feeding expressed breast milk.

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- Q6** Enumerate three specific conditions under which a mother must ****TEMPORARILY NOT**** breastfeed and ****NOT**** feed expressed breast milk to her infant, where she must wait until a physician determines her milk is safe

- Q7** State the specific clinical scenario where a mother must ****TEMPORARILY NOT**** breastfeed but ****CAN**** still feed expressed breast milk to her infant, including the required precautions and the timeline for safely resuming direct breastfeeding

Topic A: Possible Bacterial Infection

Q8 Enumerate the clinical signs that classify a young infant as having a *****LOCAL BACTERIAL INFECTION***** under the IMCI protocols

 Case Scenario:

Case 9:

A 3-week-old young infant is brought to the primary care unit. The family physician utilizes the structured IMCI guidelines to screen the infant for clinical signs of a localized or systemic bacterial infection.

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Q9 Enumerate six critical clinical signs that categorize a young infant under *****POSSIBLE SERIOUS BACTERIAL INFECTION*****, necessitating urgent pre-referral stabilization and hospital referral

Topic B: Significant Jaundice

Q10 Enumerate the specific clinical signs and thresholds that classify a young infant under *****SIGNIFICANT JAUNDICE***** according to IMCI guidelines

 Case Scenario:

Case 10:

A mother brings her 10-day-old young infant to the family medicine center because she notes progressive yellowing of the skin. The physician checks the infant's age, sclera, and limbs to determine if the clinical presentation constitutes significant jaundice.

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Topic C: Pneumonia (Cough or Difficult Breathing)

Q11 Define the specific age-based respiratory thresholds used to classify *****Fast Breathing***** under IMCI guidelines for a child aged 2 months up to 11 months, and for a child aged 12 months up to 5 years

 Case Scenario:

Case 11:

A 14-month-old child weighing 11 kg is brought to the clinic presenting with an acute cough. The child is calm, and the physician counts the respiratory rate over one full minute and evaluates the chest wall for any retractions.

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Q12 Enumerate the clinical signs that explicitly classify a coughing child under *****SEVERE PNEUMONIA OR VERY SEVERE DISEASE*****, requiring urgent pre-referral antibiotic delivery and hospital referral

Topic D: Diarrhea

Q13 Enumerate the specific clinical signs used to classify a child under *****SOME DEHYDRATION***** according to the IMCI diarrhea assessment chart

☐ **Case Scenario:**

Case 12:

A 2-year-old toddler presents with a 3-day history of frequent loose, watery stools. The child is restless and highly irritable, and the family physician performs an abdominal skin pinch test to determine the correct rehydration protocol.

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Q14 Enumerate the clinical parameters that define *****SEVERE DEHYDRATION*****, and outline the four operational flowchart steps to manage this condition if intravenous (IV) fluid access is completely unavailable nearby